

37% of Members Drive 62.5% of Cost

GROUP HEALTH RISK STRATIFICATION

A multi-variable predictive risk-scoring model of 554 insured members across Rp 4.34 billion in annual claims. Isolating high-cost segments, clinical comorbidities, and provider network concentration.

PREDICTIVE CASE MANAGEMENT

Identifies at-risk members before high-cost acute escalation.

FINANCIAL ROI IMPACT

12–18% projected claims reduction (Rp 520M+ annual ROI).

554
INSURED MEMBERS

Rp 4.34B
TOTAL ANNUAL SPEND

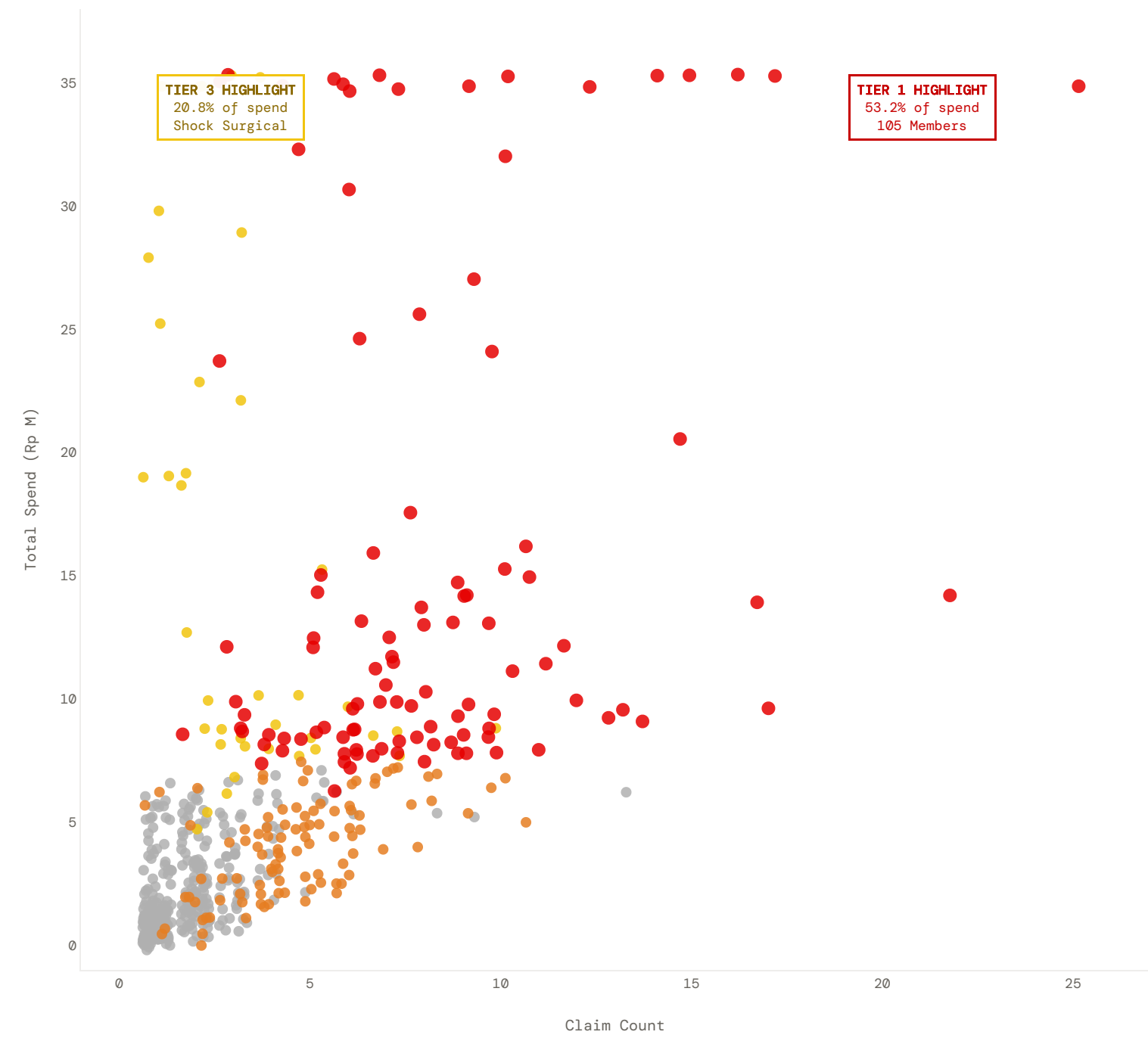
2,064
CLAIMS PROCESSED

Rp 7.84M
AVG SPEND / MEMBER

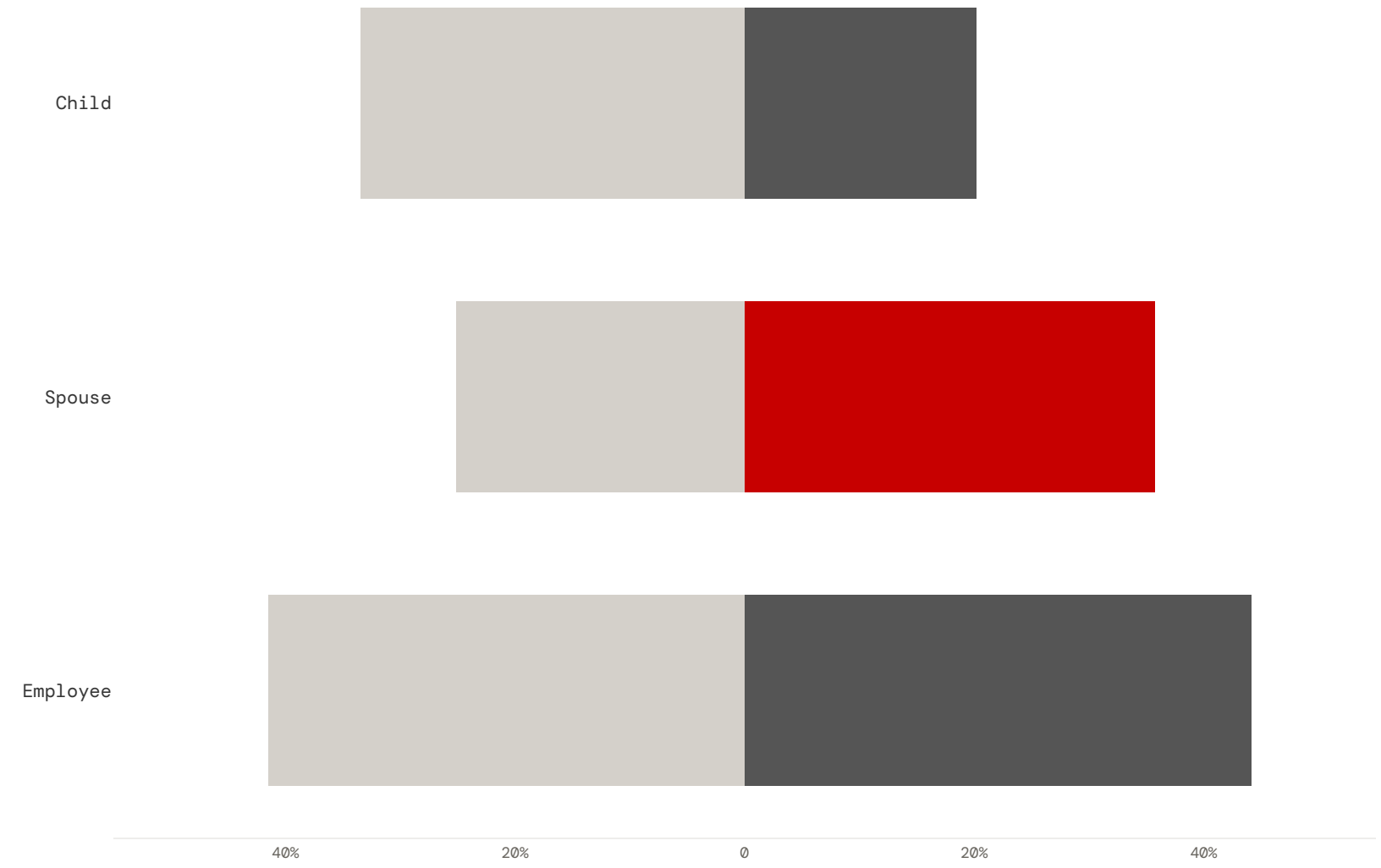
Pareto Asymmetry: 37% Members Generate 62.5% Spend

COST DISTRIBUTION

MEMBER SPEND VS. CLAIM FREQUENCY (WITH KEY TIER HIGHLIGHTS)



POPULATION SHARE VS. SPEND SHARE BY RELATIONSHIP



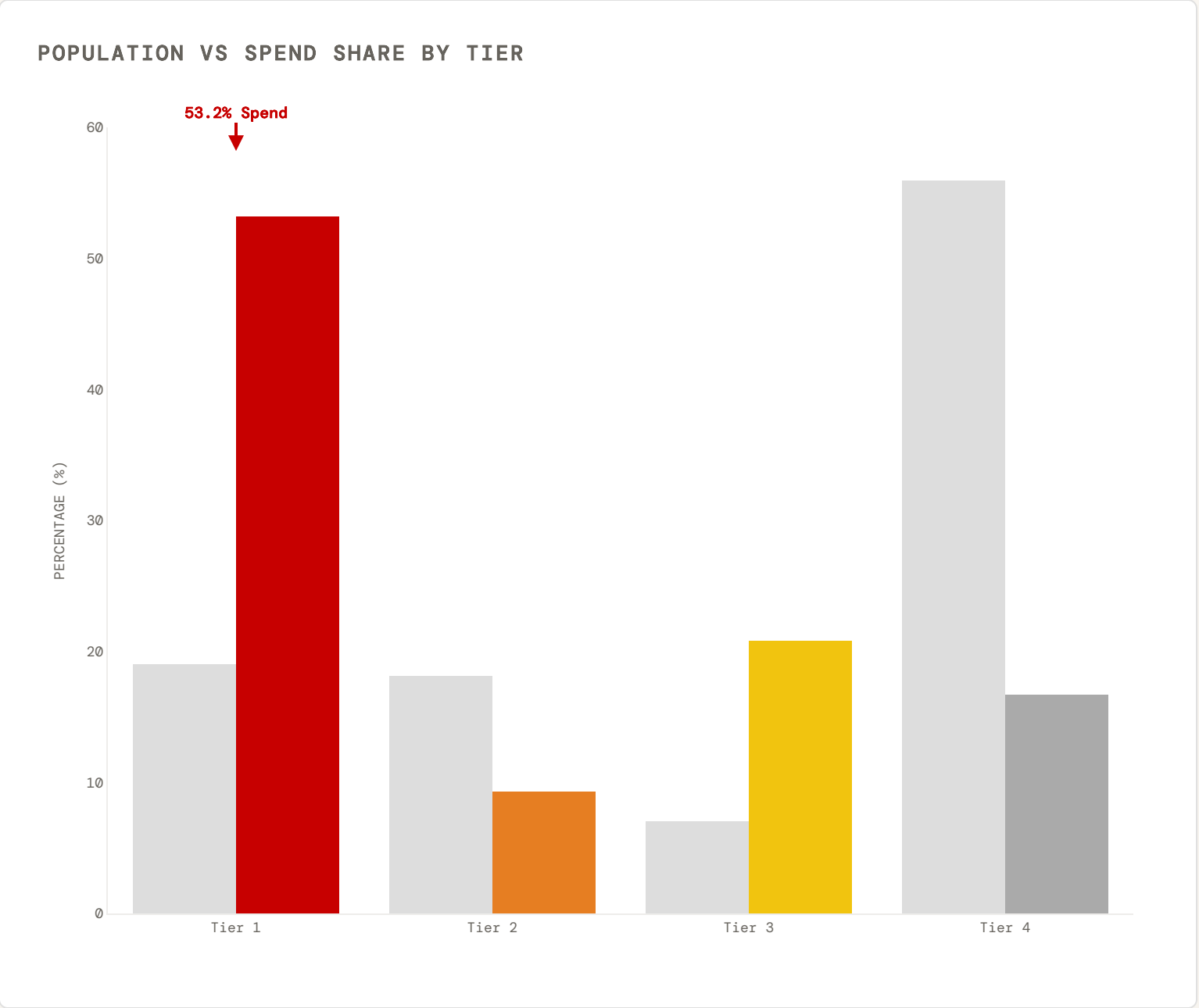
STRIKING DEMOGRAPHIC FINDING

Spouses represent only 25.1% of members but generate 35.7% of total claims cost—a per-capita ratio of 1.42× (+42% vs. plan average). Wellness outreach must explicitly cover dependents.

4-Tier Risk Matrix & Tier Cost Concentration

RISK MATRIX

	LOW UTILIZATION (<4 CLAIMS)	HIGH UTILIZATION (≥4 CLAIMS)
10% COST) HIGH SEVERITY (TOP	Tier 3 · Shock Claims 39 Members (7.0%) · Rp 903M (20.8% Cost) Avg Rp 23.1M / episode. Episodic surgical cases.	Tier 1 · High Risk 105 Members (19.0%) · Rp 2.31B (53.2% Cost) Primary Case Management target. 49 chronic cases.
NORMAL SEVERITY	Tier 4 · Standard 310 Members (55.9%) · Rp 725M (16.7% Cost) Healthy population needing standard wellness maintenance.	Tier 2 · High Utilizer 100 Members (18.1%) · Rp 404M (9.3% Cost) High frequency low-cost visits. Manage via provider steering.



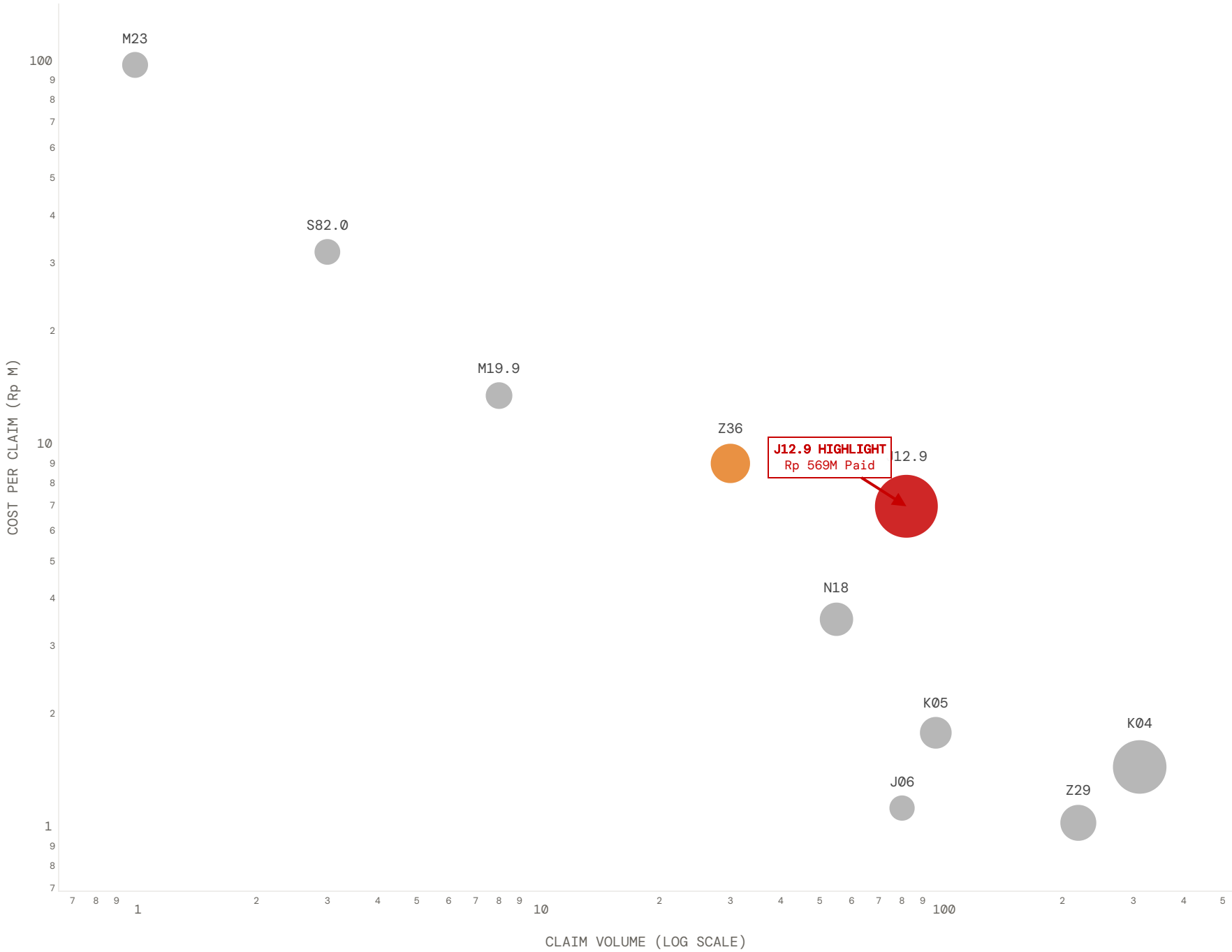
CRITICAL TIER HIGHLIGHT

Tiers 1 & 3 combined account for only 26% of members, but generate 74% of total claims expenditure!

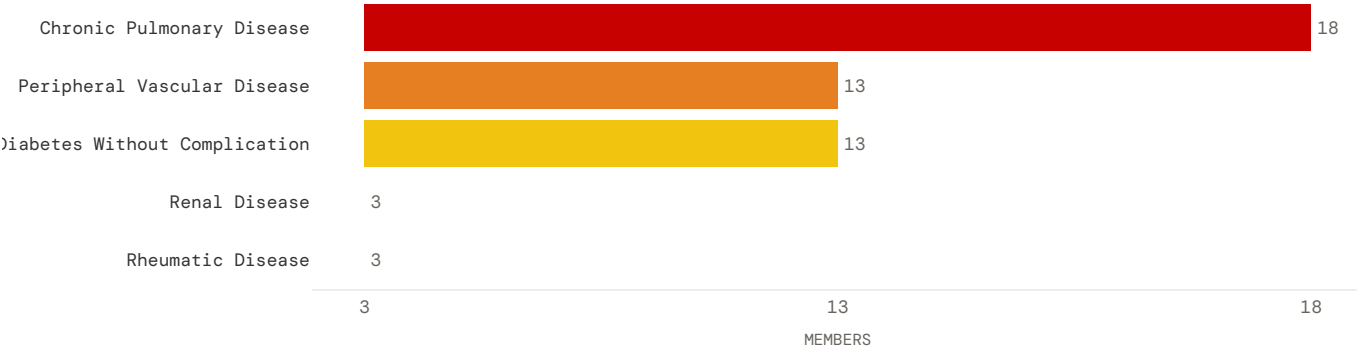
Diagnosis & Comorbidity Clustering (52 CCI-Positive)

CLINICAL PROFILE

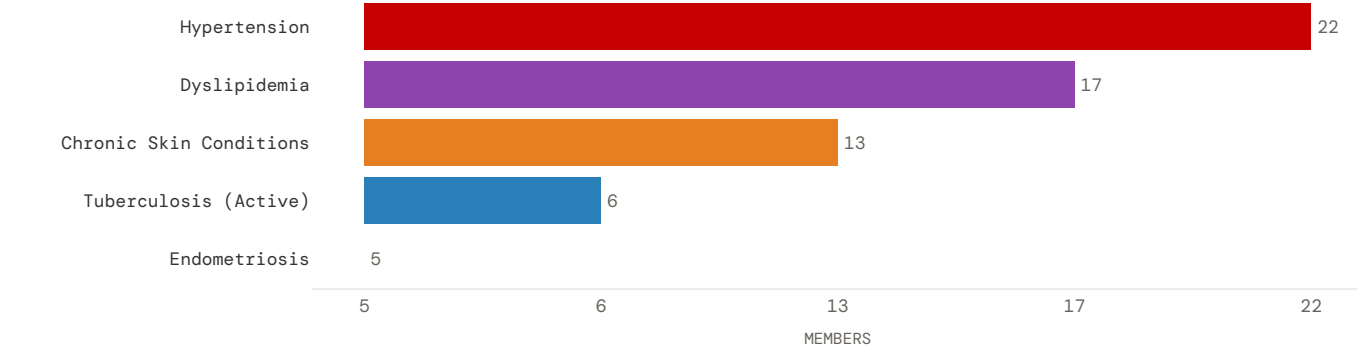
LEADING ICD-10 DIAGNOSES • VOLUME VS. COST PER CLAIM (LOG-SCALED)



TOP CCI CONDITIONS (VIBRANT COLOR CODED)



ADDITIONAL CHRONIC FLAGS



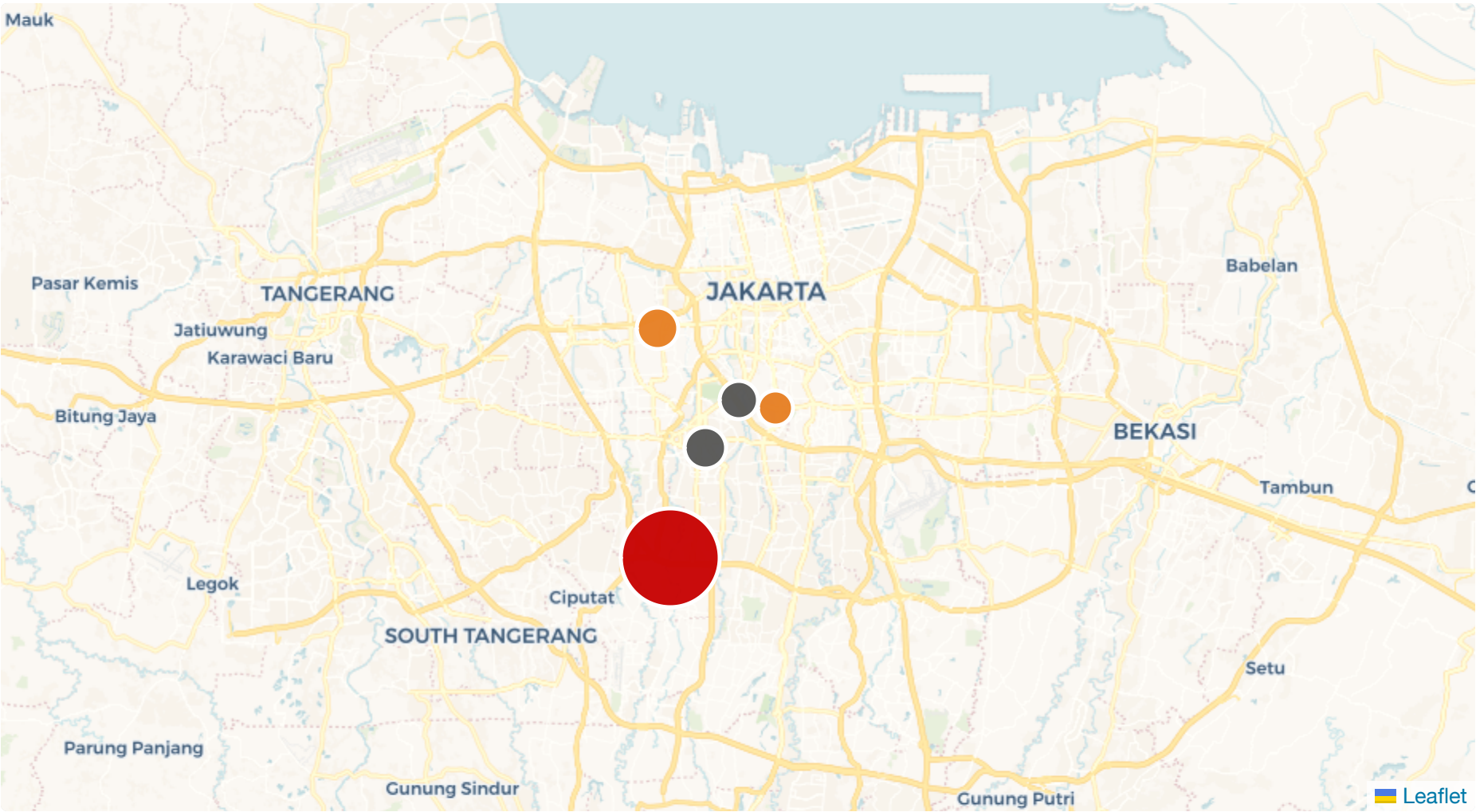
CLINICAL TAKEAWAY

Hypertension (22 members) & Dyslipidemia (17) are leading metabolic precursors to high-cost acute events.

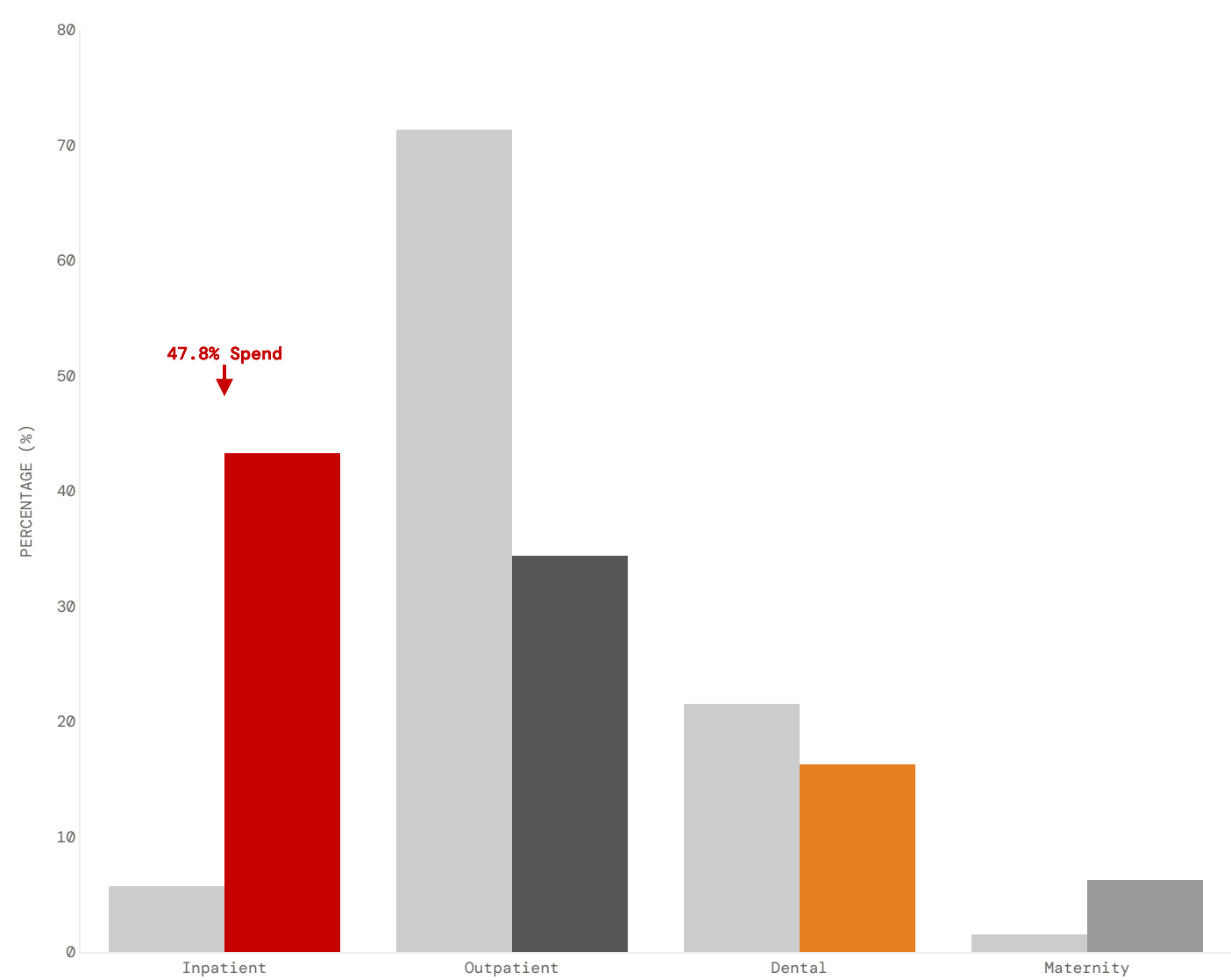
Top 5 Hospitals Control 38.3% of Total Plan Spend

NETWORK ANALYSIS

PROVIDER GEOGRAPHIC NETWORK MAP (JAKARTA METRO AREA)



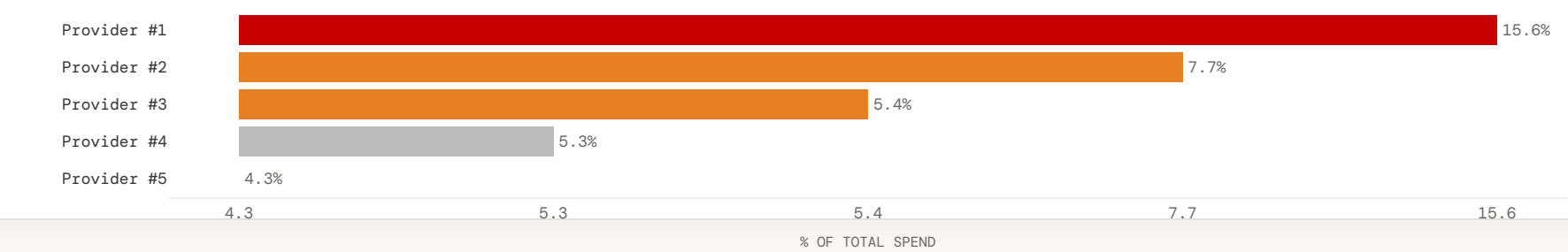
CLAIM VOLUME SHARE VS COST SHARE BY BENEFIT



INPATIENT SEVERITY & STEERING

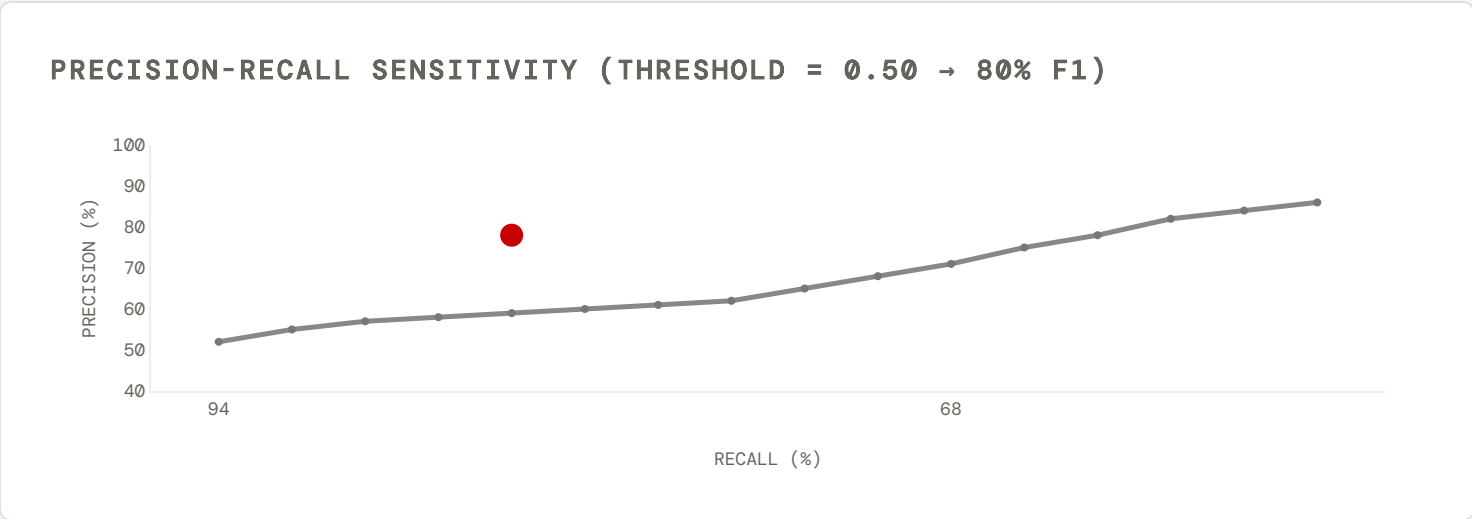
Inpatient claims represent 9.3% volume → 47.8% cost share (Rp 16.2M avg admission). Provider concentration gives leverage for fee negotiation.

TOP 5 PROVIDERS BY COST SHARE (%)



4-Phase Implementation & Projected Savings

ML & FINANCIAL ROI



PHASE 1 • 0-30 DAYS

Tier 1 Case Management

Outreach to 105 Tier 1 members & 49 chronic cases to reduce emergency admissions.

PHASE 2 • 1-3 MONTHS

Spouse Screening

Address 1.42x spouse cost ratio through metabolic disease screening.

PHASE 3 • 3-6 MONTHS

Model Recalibration

Incorporate pharmacy & biometric data to target >85% F1 score.

PHASE 4 • 6-12 MONTHS

Continuous ROI

Automated risk rescoring on new claims accrual. Measure full-year ROI.

12–18%

PROJECTED COST REDUCTION

Rp 520M+

ESTIMATED ANNUAL SAVINGS

3 : 1

ESTIMATED INTERVENTION ROI